Fortnightly cetuximab + modified FOLFOX-6

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · chemotherapy and antibody

Active medicines: Cetuximab + Oxaliplatin + Folinic acid + Fluorouracil

Emetogenic risk: Moderate

Medicine	Dose / route	When
Chlorphenamine	10 mg intravenous (IV) bolus	At least one hour before cetuximab Repeat for subsequent cetuximab infusions; assess prior infusion reactions.
Dexamethasone	8 mg IV bolus	At least one hour before cetuximab on Day 1 This corticosteroid also covers the treatment-day corticosteroid part of antiemesis where appropriate. Pharmacy must reconcile route, dose and timing; do not automatically add a second 8 mg antiemetic dose.
Ondansetron	16 mg PO once	Before Day 1 chemotherapy NCIS V8 moderate-risk row; consider QT risk, low electrolytes, interactions and the individual prescription.
Dexamethasone	8 mg PO daily	Days 2 and 3 after the administered Day 1 chemotherapy NCIS V8 delayed prophylaxis. Reconcile with any other steroid prescription; re-anchor dates if treatment is delayed.
Metoclopramide	10 mg PO up to three times daily as required	Rescue nausea treatment only as prescribed NCIS V8 rescue option; check contraindications, renal function, interactions and prescribed duration.

Additional phase-specific support

- Antiemetic classification refers to the active treatment day. Reconcile duplicate steroid entries before prescribing.
- No automatic granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) schedule is added; use the patient's neutropenia history, risk and consultant plan.
- Cetuximab premedication and infusion observation remain necessary even on an antibody-only day.
- Start local EGFR skin care; monitor magnesium, calcium and potassium before and periodically during therapy, then monthly for two months after the final dose.
- Cold exposure can precipitate oxaliplatin neurotoxicity; do not recommend ice chips during fluorouracil in this combination. Review neuropathy every two cycles and sooner if symptoms interfere with function.
- Confirm line care, pump contact details and disconnection arrangements; report chest pain, severe early fluorouracil toxicity or pump/line problems urgently.
- No ice chips during fluorouracil in this oxaliplatin regimen. Review neuropathy and cold-triggered throat symptoms; new unexplained cough or breathlessness requires urgent assessment.
- If treatment is delayed for cytopenia, follow the source-specific weekly blood-count checks and maximum delay; assess the antibody and chemotherapy components separately when indicated.
- EMA Erbitux section4.2: chemotherapy must not begin earlier than one hour after the end of cetuximab.
- Observation attribution: NCCP permits stopping routine post-infusion observation after two tolerated doses; EMA Erbitux specifies close monitoring during and for at least one hour after infusion. Reconcile the approved local observation policy; do not automate removal of monitoring.
- Medication review: NCCP contraindicates concomitant brivudine, sorivudine and analogues with fluorouracil; review dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD)-affecting medicines, phenytoin toxicity/levels and warfarin international normalised ratio (blood-clotting test) (INR).
- NCCP p1 permits cessation of observation after two tolerated cetuximab doses, whereas Erbitux SmPC section 4.2 requires monitoring during and for at least one hour after each infusion. Reconcile and document the local observation plan; do not promise early discharge.
- Folinic acid follows the bolus 5-FU hold/omission. Reconcile DPD inhibitors, DPD status and suspected fluoropyrimidine cardiotoxicity before administration.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00733 v2a · p6 · NCCP NCIS antiemetics V8 (2025), Table 1 and multi-day principles

Sources checked 2026-11-07 · SACTCheck v0.84.35 · Regimen-level drafts; prescriber and oncology-pharmacy verification required